

SPECIALTY ROUTING · AUDIT WORKSHEET

Five Clauses Decide Where Your Specialty Drugs Fill.

The PBM Owns Four.

Three audit passes. One contract clause set. One routing pattern.

This worksheet walks plan sponsors through a three-pass audit of specialty drug routing. Each pass uses the five contract terms decoded in this morning's LinkedIn post (specialty pharmacy designation, limited distribution drug access, exclusive specialty arrangement, specialty channel pass-through, specialty performance guarantee). Half a day of work surfaces the routing pattern your contract design does not predict.

01 Pull the by-pharmacy specialty fill breakdown

Open your most recent quarterly report. Find the specialty drug section. Push for a drug-by-pharmacy breakdown: for each specialty drug filled in the quarter, where did it fill, by pharmacy NPI or pharmacy name?

If the breakdown is not in the report, request it. The PBM has the data; the report format is a contract negotiation.

02 Classify each fill by pharmacy ownership

For each specialty fill, classify the dispensing pharmacy: PBM-owned (or PBM-affiliated), independent specialty pharmacy in the network, or limited distribution drug pharmacy (manufacturer-restricted).

Calculate the share by ownership category. Compare to the network design described in your contract.

03 Compare the routing pattern to the contract clauses

Open your contract. Find the specialty routing language: any clause naming the PBM-owned pharmacy as preferred, exclusive, or default; any specialty drug list attachment; any prior-authorization or step-therapy provision tied to specialty fills.

Read those clauses against the actual routing pattern. The gap is your renewal-leverage item.

04 The Pattern PBS Sees Most Often

Across approximately 100 PBM contract reviews and audits annually at PBS: the specialty routing clause is rarely as exclusive as the routing pattern. The contract preserves member choice on paper; the operational workflow steers most fills to the PBM-owned channel regardless. The leverage is in the prior-authorization and step-therapy clauses, not in the network-membership clause.

05 Paste-Ready: Send to Your Broker or PBM

DATA REQUEST • TO YOUR PBM ACCOUNT TEAM

Subject: Specialty drug fill breakdown – last four quarters

Please provide:

1. A drug-by-pharmacy specialty fill breakdown for the last four quarters, including pharmacy NPI, pharmacy name, drug name, fill count, and total plan-paid dollars per drug per pharmacy.
2. The current Specialty Drug List (Attachment C or equivalent), including any updates since contract effective date.
3. The Limited Distribution Drug access list (Schedule LDD-1 or equivalent).
4. The PA / step therapy workflow design for our top 10 specialty drugs by spend.

This data is for internal contract performance audit. We expect delivery within 14 business days.

06 This Quarter's Action Plan

STEP 1 • THIS WEEK

Pull the four-quarter fill breakdown by pharmacy NPI

Send the data request above. Confirm receipt and timeline. If the PBM cannot deliver within 14 business days, document the delay; that is its own audit finding.

STEP 2 • WEEK 2

Classify ownership and calculate concentration

Aggregate fills by ownership category. The PBM-owned share, in percentage terms, is the lead metric for the renewal conversation.

STEP 3 • WEEK 3

Read the routing clauses against the pattern

Highlight the specific contract language that authorizes (or fails to authorize) the routing concentration you found. The clause numbers go in the renewal redline.

STEP 4 • BEFORE THE NEXT QUARTERLY REVIEW

Bring the audit to your broker

Specialty routing is a recurring renewal-leverage item. Document the audit, file it, bring it to renewal.